

إِسْمُ الْمَرِيضِ رِبَاعِيًّا : رِشَا عَبْدُهُ مُحَمَّدٌ سَعْدٌ

الحي / المدينة : اوسيم

المحافظة : الجيزة

رقم البطاقة : 27302032101401 رقم التلفون : 01014104055

العمر(تاريخ الميلاد) : 03-02-1973 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمدة 6 شهور

التعليق على التشخيص : -----

الاسم: د/محمد كيلاني الوظيفة: _____
 الاسم: د/ندى حسن الوظيفة: _____
 الاسم: د/نرمين رمضان الوظيفة: _____
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جھة العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: رشأ عبده محمد سعد National ID: 27302032101401	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





Ministry of Health and Medical Services
Kingdom of Saudi Arabia

Ministry of Health and Medical Services
Kingdom of Saudi Arabia



A-Patient ID:

1- Name: *Dr. Hani Al-Faraj*

Sex:

MDT form

3- Medical numbers
Female

83149057

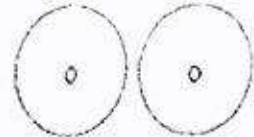
Old Mx:

Unit:

3976-2023

2- ID number: 27302032101401

Patient Age: 51 Y



Family History

Yes
No

Phone

File

01011565382

EM

ST

REASON OF MDT

Case history

Mental status

Stable

NRS Score

Stable

GROUP

ST

Date of MDT

Case history

Comorbidity

Stable

Menopausal status

Stable

Initial stage

Stable

02/01/2024

Clinical data					
Breast Imaging:			Mets Workup:		
RT: UOQ lesion at 11 o'clock 13*6 mm			+VE PALNS MRI: UOQ enhancing lesion 10*9 mm		
at 8-2 o'clock 12*18 mm			CT CAP: chest: free RT: adnexal simple cyst		
at 7*8 o'clock 16*16 mm			NUT: abdomen well lesion: Spigelian lesion		
DX IDC	ER 8/8	PR 8/8	Her 2 Neu 1	SISH	Ki67 15
T			N		
2			1		
M			0		
Sonosammography					
PATIENT HISTORY					
RT MRM: free margins					
LN 5/16					
MDT OUTCOME & PLAN					
for adj AC then Taxotere					
PORT					
ADJ AI + zoladex					
SIGNATURE					
MEDICAL ONCOLOGY	<i>Dr. Hani Al-Faraj</i>		RADIOLOGY	<i>Dr. Hani Al-Faraj</i>	
SURGERY	<i>Dr. Hani Al-Faraj</i>		PATHOLOGY	<i>Dr. Hani Al-Faraj</i>	
Psychiatric	<i>Dr. Hani Al-Faraj</i>		Nutrition	<i>Dr. Hani Al-Faraj</i>	

ent Name	: الأستاذة / رشا عبده محمد سعد	Request No.	: 13008855
ent No.	: 27302032101401	Request Date	: 05/12/2023
i No.	: 4319/23	Verification Date	: 23/12/2023
:	: Female	Financial Category	: مبادره - مبادره
:	: 50 Y	Department	: Out Patient
errefing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

anical Data: Right breast cancer, Up-Front
uture Specimen: Right modified radical mastectomy specimen.

Gross Examination:

Procedure: Right modified radical mastectomy measured 23x18x6.5 cm with covering skin
ipse measured 23x9 cm.
Tumor extent: A greyish white firm tumor.
Size: 3.5x2x1.5 cm.
Skin involvement: Nipple & areola were unremarkable grossly.
Distance from margins: 0.1 cm away from deep margin , 1.5 cm from skin .
Lymph nodes number and size: 16 L.Ns up to 1.5x1 cm.

Microscopic Examination:

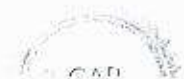
Histological type: Mixed ductal and lobular carcinoma .
Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II .
Ductal carcinoma in situ: No.
Perineural invasion: Yes.
Skin involvement: Free nipple .
Lymph node involvement: Positive lymph nodes (5/16)
Resection Margins: Free deep margin .

Diagnosis:

Right breast cancer ~~post NAC~~, Right modified radical mastectomy specimen:
Mixed ductal and lobular carcinoma , grade II .
Free deep margin .
Free nipple .
Pcsitive lymph nodes (5/16)
Perineural invasion.

Verified by :

Dr.Heba Hasan





Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي



Patient Name	: الأستاذة / رشا عيده محمد سعد	Request No.	: 13008855
Patient No.	: 27302032101401	Request Date	: 05/12/2023
Path No.	: 4319/23	Verification Date	: 23/12/2023
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 50 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

- Stage: pT2, pN2

Prof Dr. Amal Hareedy
Dr. Heba Hassan

Verified by :
Dr.Heba Hasan



Name : Rasha Abdu Mohammed Saad

Date : 26-11-2025 12:10

Branch : Dokki

Medical No. : 259095

Ref.Physician Prof : Gostaf Roseh Hospital

Ins.Company : تعاقد مستشفيات

Tc^{99m} MDPSKELETAL SCINTIGRAPHY**History**

- A 52-year-old lady, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient Name: Rasha Abdou Mohamed Saad	Date : 10/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 52 Year(s)
Medical ID: 3976/2023	Pat. ID: 46387

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS

REVEALED

History case of right MRM. In comparison to the last study dated 24-3-2025, the current study reveals:

I- CHEST:

- Right upper and middle lobar reticulonodular changes with left lower lobar atelectatic band.
- Clear right mastectomy operative bed, no definite residual or recurrent lesions.
- Unremarkable left breast with no definite suspicious lesions.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar, mediastinal or axillary masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- The liver shows homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses, diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Normal appearance of the small and large bowel loops.

III- SURVEYED SKELETON:

- Few osseous lytic lesions are noted at the left iliac bone and both femoral necks for further follow up.

Opinion:

- Few osseous lytic lesions are noted at the left iliac bone and both femoral necks for further follow up.
- Right upper and middle lobar reticulonodular changes with left lower lobar atelectatic band.
- Clear right mastectomy operative bed.
- Unremarkable left breast with no definite suspicious lesions.

Much Obligated;

Dr. Zeinab Aly, MD

Pat. Name: Rasha Abdou Mohamed	Pat. ID: 13939632
Procedure Name: MRI hips joints	Date: 01/10/2025

MR EXAMINATION OF BOTH HIP JOINTS

TECHNIQUE:

Coronal T1 & STIR WIs

Axial T1 & T2 WIs

Sagittal T1 WIs

FINDINGS:

Rather defined peripheral lesions at the femoral head and neck junctions displaying low T1 and high T2 signal and high central high T1 signal at right side likely intralesional fat.

Otherwise normal bone marrow signal of both femoral heads. They showed normal size with intact regular cortical outlines.

Preserved hip joint spaces with smooth regular articular surfaces.

No MR evidence of synovial thickening or effusion.

Intact peri-articular muscles with preserved fat planes.

OPINION: -

Picture suggestive of bilateral fibrocystic changes (Synovial herniation pits) at the femoral head and neck junctions, more evident at the right side, for clinical correlation.

Dr.Abeer Muneer,MD



Patient Name: Rasha Abdou Mohamed Saad	Date : 10/05/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 52 Year(s)
Medical ID: 3976/2023	Pat. ID: 46387

BILATERAL BRESATS US

Indication:

Patient had right MRM

Technique

- Bilateral ultrasound

Report:

- No left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Right mastectomy bed small seroma measured 2.4x1.5 cm, no evidence of recurrent mass lesions
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with operative bed small seroma yet no evidence of recurrent mass lesions or pathological; regional LNs
- No left breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. basma mohamed

10/05/2025 1:40 PM



Patient Name: Rasha Abdou Mohamed Saad	Date : 06/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 52 Year(s)
Medical ID: 3976/2023	Pat. ID: 46387

Left BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- scattered fibroglandular densities of the left breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions, with small axillary tail operative bed seroma, measured 2x4cm
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Engy Abdallah

06/03/2025 11:02 AM



Patient Name: Rasha Abdou Mohamed Saad	Date : 30/12/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 52 Year(s)
Medical ID: 3976/20523	Pat. ID: 46387

TARGETED RIGHT CHEST WALL US

Report:

- Right axillary seroma with maximum thickness of 3 cm.
- Free right operative bed with no evidence of recurrent mass lesions
- Free right supra and infra regions.

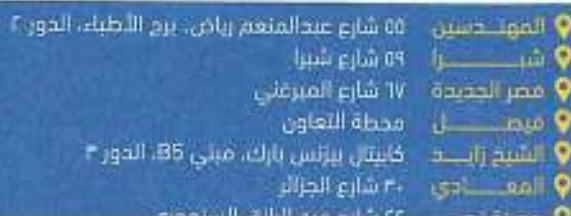
Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions yet with axillary seroma as described.

Yours truly

Dr. Mohamed Mohsen, M.D.

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the mid-thigh to the base of the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- **Liver showed average SUV ~2.2**
- **Liver showed max SUV~ 2.8.**



Visit Number: 61124503291

Client Name:

Patient Name: **Rasha Abdo Mohamed**

Referred By Prof: **Sherif Shaban**

Age / Sex: 51 year / Female

Exam Date: 13 Feb 2024 00:00

Patient ID: 17944321 Acc: 891613

Printed: 14/02/2024 07:16:27 PM

PT

REPORT: -

Review of the PET, CT and the fused images revealed: -

I. Loco regional (breasts and axillae):

- **Right breast:** Status post MRM and axillary clearance showing clear operative bed with no metabolically active (FDG avid) residual/recurrent mass lesions detected.
- No metabolically active (FDG avid) loco-regional /axillary lymph nodes.
- Low grade metabolically active operative bed mild subcutaneous fat stranding and hypodense collection / seroma, measuring about 3.7x2.6 cm, with max SUV 2.2 (**post-operative sequelae**).
- **Left breast** is clear showing no metabolically active (FDG avid) lesions.
- No metabolically active (FDG avid) Left axillary lymph nodes.

II. Rest of the body:

- **No metabolically active (FDG avid) lesions detected all over the surveyed body that would account for nodal or distant metastatic deposits.**
- **Osseous:** Mild increased FDG uptake by the axial skeleton, yet no lytic/ sclerotic lesions, likely secondary to anemia/cytopenia for correlation with CBC.
- **The liver:** average sized liver, showing normal FDG uptake with no metabolically active (FDG avid) hepatic focal lesions.
- **The spleen:** is average in size showing normal FDG uptake with no metabolically active (FDG avid) splenic focal lesions.
- **Lungs:** Clear both lung fields. No metabolically active pulmonary masses or nodules.

SIDE FINDINGS:

- Borderline cardiac size, for Echocardiography.
- Sacralized L5. Degenerative spondylolisthesis of L4 over the sacralized L5.
- Non FDG avid benign bone cyst of iliac side of left sacroiliac joint.
- Small wide-neck right inguinal hernia.
- Left-sided Port-A-Cath.



 جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 رشا
 عبده محمد سعد
 ٢٤ من الجيش
 مركز اوسيم - الجيزة
 ٢٧٣٠٢٠٣٢١٠١٤٠١
 K83572143

Dms



 ٢٠٢٣/٠٦
 ٢٧٣٠٢٠٣٢١٠١٤٠١
 بدون عمل
 أفتى
 مستلمة
 البطاقة سارية حتى ٢٤/٠٦/٢٠٢٣
